Supplementary Table 3: Summary data extraction: Group processes

No	Author year	Who is a PWG member?	Context: community, cultural, political institutional factors	PWG intra group relationships	Group processes; structure, facilitation, activities, processes, decision-making, reflexivity.
1	Lowell, A., Kildea, S., Liddle, M., Cox, B., & Paterson, B. (2015)[6]	All pregnant women in the community potential participants. Also, young women and school children	**Community/ cultural** Program of Strong Women, Strong Babies, Strong Culture Program (SWSBSC) is about Aboriginal cultural underpinning of maternity work. Acknowledgement of poor living conditions and poverty affecting women. **Institutional/political** Grant funding ceased in 2008 and program taken over directly by state government health department (NT Department of Health) and lost some of its direction.	Value of two-way cultural learning for Aboriginal and non-Aboriginal staff noted. Unclear about the effects on the participants in terms of relationships.	**Structure** Generally structured around the needs of the community and skills and interest of coordinators. **Facilitation** Two program coordinators one in top end and one central. Strong women workers (SWW) in communities. **Activities** Varied including culture camps, playgroups and Early Childhood Services, health related education sessions at schools, working with midwives in two-way learning. Many cultural activities that were "hidden" to the health service were undertaken including smoking ceremony for new babies. **Decision-making** In some communities locally SWW coordinators devise and run all activities – some input from midwives and health staff. At a higher-level unclear decision-making. **Reflexivity** Authors state researchers had extensive collaborative research experience with Aboriginal communities
2	Earle-Crane, M. (2000)[24]	Women at risk for poorer birthing outcomes and from low-income families who are socially or economically disadvantaged	**Institutional/political** Funding obtained through Health Canada Prenatal Nutrition Program for 9 Healthy Baby Clubs in designated regions of Newfoundland. Noted that having access to transport and child-care enabled attendance.	Relationships that were supportive between participants and resource mothers were reported as one of the major outcomes. So much so that participants wanted the groups to continue.	**Structure** Structured pre-natal group education with peer support. Resource mothers provide emotional support and keep in touch with women. Nutritionists and public health nurses provide education and food supplements. **Activities** Structured education sessions and resource mother support at and between meetings. Transport provided and child-care. **Process** Conducive atmosphere. Group process involved giving and receiving support and this was highly valued. Some participants wanted more input into planning the group meetings. **Decision-making** Decisions made by staff. Members wanted more input into decision-making **Reflexivity** Author notes limitations of some HBC workers understanding the situation of participants
3	USAID & Burma Shae Thot (2018)[48]	Residents of villages and towns in three Burma regions Mother's groups may	**Community/culture** Different degree of community unity across communities and differing leadership capacity. Differing levels of	Unclear- not reported on	**Structure** Shae Thot (program) established sub-committees (like WASH committees or Mothers Groups), that were responsible for implementing sector-specific activities. The VDC acted as a central coordinating body of these 17 sub committees of volunteers and was the cornerstone of the community's civil society. **Facilitation/Activities**

		have only involved women - unclear	community engagement and different Village Development Committee (VDC) working. **Institutional/political** Political instability and climate change, armed conflict in Kayah state. Lack of overall project plan, funding issues and governance issues in the groups and sponsors.		Volunteer health workers called “Change Agents” were trained in safe pregnancy practices, diagnosing and treating common illnesses, and facilitating emergency care. These Change Agents were linked to Mothers Groups, networks of mothers who met weekly to learn about and discuss MCH-related illnesses, hygiene, and nutrition. Village Development Funds were provided. **Decision-making** Unclear not reported on **Reflexivity** Authors see persistent traditional beliefs and practices as a barrier to integration with health services.
4	Saville, N. (on behalf of MIRA) Centre for International Health and Development (UCL)[35]	Unclear about how members were selected and whether the groups were pre-existing.	**Community/culture** Meetings addressed local nutritional issues particular to the Maithili-speaking ethnic group. Program material used was reported to be culturally sensitive.	Unclear – not reported on	**Structure** 270 women’s groups in 30 VDCs. **Facilitation** FCHVs assisted by a literate “co-facilitator group member “to facilitate the group”. Each FCHV and co-facilitator was paid Rs200 (A$2.50) incentive per meeting. PLA **Activities/Process** Issues identified by the groups about nutritional beliefs and practices. There were planning meeting topics. Groups decided what strategies to implement. Then 5 main nutritional-related topics introduced through a story based upon local practices picture card. Social dramas. Also, home visits and education. IYCF visits to targeted women with children around 6 months. **Decision-making** Reported that women made decisions about strategies to implement. **Reflexivity** Unclear
5	Ministry of Health (Sri Lanka) and UNICEF Sri Lanka (2015)[49]	Community women were members of the women’s group	**Community/culture** Culture limited women joining groups. There was not a capacity building atmosphere in the traditions that were not perceived to be supportive of MCH. **Political/institutional** Conflict between North and East of the country seeing fluctuations in nutrition status, food insecurity, poverty, and emergence of female/child household heads. Community, prevalence of Domestic Violence, and marginalisation of unwed	Qualitative interviews reported that women joined new networks through the groups and strengthened unity and trust among community members	**Structure** Groups were existing but were in a winding down phase. It was the “change agents” who connected with community women and were linked to mother’s support groups (MSGs). **Facilitation** Groups run by volunteer Health Workers “Change Agents” who were trained in safe pregnancy practices and diagnosing and treating common illnesses. Also encouraging the use of health services. It was a service provision rather than an empowerment approach as there was not the participatory decision-making cycle. **Activities** Some food preparations sessions were conducted. **Decision-making** Unclear whether made decisions about content or “Change Agents” delivered content they were trained in. **Reflexivity** Unclear

			mothers. Presence of cultural and religious.		
6	Roy, S. , Mahapatra, R., Rath, S., Bajpai, A., Singh, V., Rath, S., . . . Prost, A. (2013)[43]	Village women became part of the mothers' groups.	**Institutional** Two relevant government programs (1) 820,000 Accredited Social Health Activists (ASHA) who provide services but were not trained in home-based neonatal care until 2012. (2) Conditional cash transfer scheme to increase the number of institutional deliveries was introduced.	Unclear – not reported on	**Structure** Mothers' groups were run in association with Ekjut NGO. Facilitators had numbers of groups, coordinating an average of 15 meetings per month with a population cluster of around 6,000. **Facilitation** Local female facilitators (not a health worker) were supervised by the project team. The facilitator guides women's groups through a cycle of activities involving participatory learning and action. **Process** Groups went through cycle of 4 meetings – identification of local problems, prioritising, implemented strategies, evaluation, and translation. Zone 2 was different because evidence was available about greatest risks for babies (keeping warm) and so groups focused on this. Each of the groups came up with, and implemented, different strategies. **Decision-making** The phases of the mothers group include decision-making and prioritising. **Reflexivity** Unclear
7	Gill, K. (1999)[50]	NGO selects Link worker and trains them. Link volunteers are responsible for several houses. Women then are invited to meetings	**Community/culture** Explicit acknowledgement of cultural difference, gender, need for all female groups. **Political/Institutional** Tension and violent conflict between Hindu, Muslim and other minorities Early childbearing, poor status of women and female illiteracy, poor social development in the slum. History of efforts for slum improvement. World Bank, urban health posts and Ministry willing to support autonomous action of NGOs and women's groups. A changed environment enabling autonomy.	Women formed supportive relationships through the groups. Women leant a lot and women gained courage to go out and talk to people. Women felt proud of the improvements they made in the slums.	**Structure** Monthly women's (health) groups held in the slum and were existing groups. Monthly meetings held with urban health posts to liaise. The groups were the link with the urban health post. **Facilitation.** The groups are formed by Link community volunteers (training provided) and volunteer Community health workers. There is a community development approach. A history and existing leadership were the reasons given as to why this initiative worked well in Hyderabad. **Process** The groups discuss issues, prioritise and take action across health and wellbeing with relevant agencies. Information about issues on the ground is fed into government health services. **Activities** Health promotion, vocational training, social clubs, environmental sanitation. **Decision-making** NGOs and WGs autonomous in decision-making in program and financing activities **Reflexivity** Unclear
8	Ndirangu, G., Gichangi, A., Kanyuuru, L., Otai, J., Mulindi, R.,	Young women less than 30 who had given birth in the last 2 years were	**Cultural/community** Mention of gender –based violence. Noted that characteristics of women and factors affecting	Unclear	**Structure** Young mother's clubs were run through Jhpiego to share experiences and solutions to issues while receiving health education from health facility staff and community health workers. Groups were established for this purpose. **Facilitation**

	Lynam, P., . . . Archer, L. (2015)[25]	invited to attend special groups to test the effectiveness of education materials	implementation of interventions in Nairobi's informal settlements may differ substantially from those of urban poor elsewhere.		Groups were facilitated by community health workers with support. **Process/Activity** Training sessions were structured on set topics teaching about aspects of MCH (Focus on postpartum haemorrhage (PPH), positive behaviour around childbirth, and family planning). **Decision-making** The study was defined and the groups had a set curriculum and before and after quiz about knowledge gained. **Reflexivity** Unclear
9	Gram, L., Skordis-Worrall, J., Manandhar, D. S., Strachan, D., Morrison, J., Saville, N., . . . Heys, M. (2018)[36]	Inclusion in original trial was pregnant married women between 15 and 49. This study drew on 66% of the cohort of women who participated in the original trial.	**Contextual factors** Not discussed	Unclear	Information reported about the original study 2001-3 as this study did not establish groups. Reworked women's groups. **Structure** Community-based PLA women's groups with monthly group meetings, in which members explored issues around pregnancy, childbirth and newborn health. **Facilitation** Provided by a local lay facilitator conducting groups. **Decision-making** Participatory decision-making in the original study. **Reflexivity** Unclear
10	Roche, M. L., Ambato, L., Sarsoza, J., & Kuhnlein, H. V. (2017)[26]	Guide Mother (volunteer community selected) chose mothers with children 2-5 years to attend cooking sessions.	**Cultural/community** Women in the maternal land of Tungurahua and their connection with culture and the food system was the basis of the study. Mink'a is a system of community collaboration for common good and food has social and cultural value for communities. The objective of the project was to reintroduce Quichua culture and traditional foods. Elders identified local foods and their acceptability, including reintroducing two wild leafy greens.	Social contact continued after the cooking group. "What I liked most was the union between the ladies that we formed while preparing food for all of our children".	**Structure** Asset based intervention bringing small groups of women together to cook in a "Guide" mother's house. **Facilitation** Guide Mothers were community selected, trained in participatory work from the project coordinator (World Vision) and community nutrition and health specialists. Guide Mothers were to encourage use of language and culture. **Activities** Elders attended to pass on culture, support and language which had fallen away. Knitting was reintroduced as well. **Decision-making** Women made decisions including Elders and Guide Mothers. **Reflexivity** Unclear
11	Kruske, S., Schmied, V., Sutton, I., & O'Hare, J. (2004)[51]	Women with babies up to 8 weeks of age	**Cultural/community** Increasingly midwives and nurses are adopting more client directed groups where participants set their own agenda.	EBP Groups promoted relationships women learnt from each other as well as	**Facilitation** Early Bird program support group facilitated by child and family health nurses and offered to families of infants. Midwives were trained to adopt a developmental style (non-didactic) **Structure/activities** No set agenda, baby weighing and developmental assessments available before and after group.

				nurses. Peer support, and listening to others resulted in increased confidence.	**Decision-making** The EBP approach empowered mothers as a group by de-emphasising the power of the professional **Reflexivity** There was an explicit statement that none of the nurses were to position themselves in a position of "teacher" as this made participants passive recipients and reinforced medical hegemony
12	O'Rourke, K., Howard-Grabman, L., & Seoane, G. (1998)[27]	Women but unclear inclusion criteria and whether existing Mothers Clubs or set up new ones. There was a mix of women's groups in communities already.	**Community/culture** 50 communities participated and they varied greatly in traditions and demographic characteristics. Some challenges to local community health workers as women in leadership positions did not fit easily in these roles in either traditional or western medicine.	Unclear not reported on.	**Structure** Warmi project was conducted in 50 communities in Inquisivi. Project involved study personnel organising, forming, or strengthening women's groups. **Facilitation** External study personnel primarily ran the intervention. Study team consisted of 5-6 people including 2 Save the Children nurses. Each study team met monthly with the provinces women's groups together. **Activities/process** Women's groups identified problems, worked out a way to implement a formal action plan (called autodiagnosis) training husbands and birth attendants on safe birthing. Each community identified a different set of problems, but all the groups addressed certain objectives. All received core training in set curriculum MCH and family planning. Literacy and credit schemes were established in mother's groups as well. **Limitations** Reported that local community health workers had difficulty in leadership positions. **Decision-making** Unclear but study personnel conducted autodiagnosis with group members had input to decide priorities **Reflexivity** Unclear
13	Manandhar, D. S., Osrin, D., Shrestha, B. P., Mesko, N., Morrison, J., Tumbahangphe, K. M., . . . Members of the MIRA Makwanpur trial team. (2004).[37]	**Membership** Women were enrolled in the trial if they were married, between 15-49 years and potential to become pregnant.	**Community culture** Female facilitators were necessary. **Political/Institutional** The country was at war during this trial and one district excluded at baseline for security reasons. Security problems in the district escalated during the third year of the study. Meetings had to be postponed several times in two clusters in intervention and two in control.	Unclear, except that the intervention seemed to be acceptable: 95% of groups remained active at the end of the trial despite no financial incentives and the opportunity costs incurred by women spending time away from other tasks.	**Structure/process** The facilitators recruited for this trial worked in association with the Female Community Health Volunteers. Ten meetings had a formal structure with a different meeting aim for each meeting. There were community meetings held to inform community. One supervisor for every 3 facilitators and there was a manual. **Facilitation** The facilitator was locally recruited with no health background. The facilitator supported groups through an action-learning cycle in which they identified local perinatal problems and formulated strategies to address them. Facilitators had training in health once they started. Some groups set up by local female community health volunteers already existed, but their activity was sporadic. The role of the facilitator was to activate and strengthen groups and support them through an action research cycle. **Activities** Included health system strengthening in all the cohorts **Decision-making** Women in the women's groups made decisions about their strategies and all the groups had different strategies. General learning about MCH was overall but there was no agenda about which strategies were to be followed. **Reflexivity**

					Unclear
14	Quigley, P., Green, C., Soyoola, M., Kureya, T., Barber, C., & Mubuyaeta, K. (2018)[52]	The membership of Safe Motherhood Action Groups (SMAGS) unclear. Both men and women in community discussion groups	**Community/culture** Domestic violence in communities **Institutional** There were existing Safe Motherhood Action Groups (SMAGS) due to a 2003 initiative by the Ministry of Health using community-based volunteers. More Mobilising Access to Maternal Health Services in Zambia (More MAMaZ - program reported in the paper) worked with District Health Management teams to scale up coverage of interventions.	Unclear	**Structure** Project staff member managed the program in association with the District health management team (government). Volunteers trained by project staff to provide information to SMAGs. Safe Mothers Action Groups and more broadly to community. Process of SMAGS unclear. F**acilitation** Each trained volunteer engaged community in four discussions on safe pregnancy and delivery and neonatal care. **Activities** Four community discussions about maternal and child health and the need for skilled birth attendants were delivered in communities. **Decision-making** Unclear within the SMAGS but at community level men and women reportedly made decisions about strategies needed **Reflexivity** Unclear
15	Skordis, J., Pace, N., Vera-Hernandez, M., Rasul, I., Fitzsimons, E., Osrin, D., . . . Costello, A. (2019)[38]	No new groups established for this study. Original trial women were enrolled if they were married, between 15-49 years and potential to become pregnant.	**Community/culture** Family networks are distinguished between those of the wife and those of the husband's family because women usually live with husband's family. Ethnic groups described fully and their involvement in community life and care seeking behaviour.	Not available – no groups established for this study.	**Process** No groups established for this study. No details about the original groups except that they were established as a medium for health information which is only partly correct. **Decision-making** Not available **Reflexivity** Unclear
16	Tripathy, P., Nair, N., Barnett, S., Mahapatra, R., Borghi, J., Rath, S., . . . Costello, A., (2010).[44]	Women 15-49 living in the chosen clusters and who had given birth during the study period (2005-8)	**Community/culture** Acknowledgement of culture/caste makeup of the district. Adivasi (Indigenous people) and scheduled castes were included in the PWG. There were different languages spoken and high levels of poverty and illiteracy in the regions where the studies were conducted. **Institutional**	Not reported on in this paper but researchers hypothesized that the large decrease in maternal depression (year 3) might have come about due to increased problem-solving skills and	**Structure** This study was conducted by a similar team to the Makwanpur team and used the same methodology and training materials for the facilitators. In the intervention arm, a facilitator was selected by community who could travel to groups and could speak local language. Strong supervisory program and training for the facilitators was seen as essential. **Facilitation** Facilitator worked with 13 groups meeting once a month using PLA cycle. Discussion was about MCH problems and then implementation of strategies. Education provided through games about delivery and cleanliness. **Activities** Appreciative enquiry workshops for front line government health workers in the area. Village health committees were formed and women PWG members gave feedback about government health services and learned more about them.

			Accredited Social Health Activists program operating.	improvements in social support.	**Decision-making** A PLA cycle was followed **Reflexivity** Unclear
17	Damtew, Z. A., Karim, A. M., Chekagn, C. T., Fesseha Zemichael, N., Yihun, B., Willey, B. A., & Betemariam, W. (2018)[53]	Each Women's Development Army (WDA) leader had 5 neighbouring households to join together and then 6 amalgamated to a network. Presumably, everyone in the household could be involved.	**Institutional** Ethiopian government introduced the Health Extension Program with a health post and two Health Extension Workers (HEW) in each community. The Women's Development Army (WDA) was also established by government.	Unclear – not reported on	**Structure** Five participating neighbouring households brought together by 1 WDA and then 6 of these groups brought together in a network. High density was one WDA to 40 households and low density was 60 households. There were set programs that were developed and delivered. **Facilitation** WDA work in association with government HEW and they share knowledge about MCH and empower one another. No information about the group processes. **Decision-making** Unclear, but reported that the intention was to empower women and encourage participatory action. **Reflexivity** Unclear
18	Turan, J. M., Say, L., Güngör, A. K., Demarco, R., & Yazgan, S. (2003)[45]	Pregnant women and expectant fathers were recipients of the activities established by the Community Design Team (CDT). CDT were female volunteer community members and health professionals.	**Community/culture** Authors stress importance of an accurate understanding of the community, including needs, resources, social structure and values.	Course participants developed support networks and advocated for better perinatal health services. Pregnant women who attended the course together continued to support each other through pregnancy and after the birth.	**Structure** Ten step structured process 1. Situation analysis in local community 2. Establishment of the Community Design Team (CDT) (All women CDT) 3. Review of data on maternal and child health 4. Identify priority needs and audience 5. Define and understand the target audience 6. Develop educational and behavioural objectives 7. Develop messages and strategies 8. Construct program plan 9. Implement plan 10. Evaluate and refine program. **Facilitator** Project officer trained local community members (CDT) who then engaged with young women **Decision-making** CDT made decisions regarding group activities, identifying community health issues and designed and evaluated interventions. Now CDT running all aspects of groups. **Reflexivity** Unclear
19	Bolton, M., Moore, I., Ferreira, A., Day, C., & Bolton, D. (2016)[28]	Contact was made with pregnant women or with children under 2 and invited to take part. Only one group	**Political context** Overall UK National Institute for Health and Clinical Excellence (NICE) points to importance of community engagement.	Women formed their own social support network through the groups, planned their own meetings	**Structure/process** Unstructured community development approach with no prior determination of what should happen. The community-led 'intervention' that evolved comprised mothers meeting together to provide mutual social support, choosing for discussion topics, concerns and worries, and sharing advice, these sessions being supplemented by requested health information and educational workshops. **Facilitation** Professional community organisers and volunteer community leaders worked to seek out mothers.

		studied with 15 members.			**Decision-making** Groups made decisions and implemented plans. They called on support when they needed to **Reflexivity** Unclear
20	Wong, M.L., Chen, P.Y.C. (1991)[46]	18 women who were mothers between 20 and 35 were trained initially in the survey methods. Through a community development process other women and men became involved in running the feeding program and the kindergarten.	**Community/culture** Social structure ranked women below men and usually women did not make decisions. Male chair necessary to gather support for the initiatives and head man's permission needed for women to be trained. 450 people housed in apartments in the long house and social relationships characterized by harmony, reciprocity, and cooperation. **Political/institutional** Community was agrarian based, hill rice paddies, fishing hunting and fruit gathering. Economically depressed.	Probably women knew each other but working together was described in preparing food for the kindergarten, fund raising with cake stalls, management committees, planning etc.	**Structure/process** With participatory facilitation, 18 trained women prepared simple questionnaire re health and feeding, collated the problems and worked with community elders to prioritise problems. Local women may have particular skills and capabilities and more accurate views than external agents about the feasibility and viability of projects. Representatives from government agriculture, health, and community development agencies formed an advisory committee to provide technical support. **Facilitation** Facilitators of the project gained an understanding of the Berawan concept of health and prepared the community for decision-making by women. Facilitators worked within community structures to sensitize men about the potential of women. Success was largely attributable to an active participatory approach, pre-implementation planning, and facilitation. New groups established. **Activities** Women set up feeding program and kindergarten and formed a health committee to oversight this. The kindergarten had support from a nearby school. Self-funded programs so everyone involved in funding and men made furniture for the kindy. **Decision-making** Women were involved in every aspect of planning and decision-making. **Reflexivity** Unclear
21	Rath, S., Nair, N., Tripathy, P. K., Barnett, S., Rath, S., Mahapatra, R., . . . Prost, A. (2010)[47]	Local women	**Culture/community** In both Jharkhand and Orissa, Adivasi groups have distinct identities and safeguard their social institutions and ancestral territory. Illnesses were attributed to supernatural causes and local diviners/ private practitioners were used to deal with health problems in pregnancy and newborns. Villages were in hilly regions with difficulty in accessing health services. **Institutional political** Numbers of NGOs and government agencies were involved in MCH including	Unclear – not reported on	**Structure** The intervention involved a participatory learning and action cycle of 20 meetings adapted from Bolivia, and Nepal. Materials for meetings included the Ekjut team's own innovations. Existing groups used. During the trial period, 244 women's groups met monthly within groups of 15-20 to discuss problems related to pregnancy, childbirth, and the post-natal period. There were four phases – identify and prioritise problem; plan strategies; implement. Although intervention cycle lasted 3 years rather than 22 months due to festivals etc. **Facilitation** Local, literate married women, (not health educators) were selected by senior Ekjut team. Facilitators had supportive families and could travel independently to meetings. They were trained in participatory communication to discuss health problems during pregnancy and childbirth in the groups. **Decision-making** Group meetings emphasised collective problem solving **Reflexivity** Unclear

			the government community-based volunteer cadre, the Accredited Social Health Activist (ASHA).		
22	Pant, P. R., Budhathoki, B., Ellis, M., Manandhar, D., Deave, T., & Mytton, J. (2015)[39]	Women ranging in age from 20s to 50s	**Culture/community** Local Female Community Health Volunteers (FCHVs), and women's group chairs, needed family support to attend meetings. Many (80%) of them had limited time attend and meeting times adjusted during labour peak times; cultivation, harvesting or processing of crops.	Unclear – not reported on	**Structure/process** Established women's' group members worked together to develop program to cover a number of common unintentional injuries in children. A manual, posters, resources for collecting feedback on the programme and parent-reported injuries in the children were developed. The FCHVs convened 10 women's groups to run over 6 months with 24–29 mothers attending each meeting (290 mothers participated in total). **Facilitation** Female Community Health Volunteers (FCHV) act as facilitators for established monthly women's group meetings. **Decision-making** Women attending groups had demonstrated that they could understand risks and identify options to minimize risks. **Reflexivity** Unclear
23	Nishiuchi, M. (1985)[54]	Women at least 20 years of age and a resident of Suita	**Political/ Institutional** Post WWII Japan and two weeks after formation of the club Korean War broke out. Work of health centre not well known – viewed as a place for TB tests only.	Unclear –not reported on. This is a narrative of the program's success stories	**Structure** Highly structured –district reps chosen by ballot among women of the same neighbourhood. Chapter officers (covering elementary school zones and include a number of towns) are chosen by district representatives and the chapters list the ideas of their members, communicate the club's principles, and conduct activities. **Facilitation** Strong engagement and community driving –working alongside health system but also finding solutions e.g., trachoma examinations in community provided by health professionals working at a temple in the evenings. Club staff provided education. Due to success this program was taken on by the city. **Activities** Six sections of the club – general affairs; MCH services; Preventative health care section; Public Hygiene; Lecture (education); Culture. Areas of focus are decided on by looking at public health data at that time. **Decision-making** Highly structured with club representatives presenting views. **Reflexivity** Unclear
24	Morrison, J., Osrin, D., Alcock, G., Azad, K., Bamjan, J., Budhathoki, B., . . .	Community women from all socio-economic backgrounds	**Cultural/ community** Cultural context varied: In Bangladesh, 80% of the population were Muslim, In Nepal, 65% were of marginalised Tamang Buddhist ethnicity. In the	Women's groups were effective in strengthening or creating social networks, Women learned	**Structure/process** Women's groups were led through PLA cycles related to maternal and newborn health. The groups identified and prioritised problems associated with pregnancy, delivery, and the newborn period, and together with communities, they planned and implemented strategies to address these problems. Women's groups addressed a knowledge deficit in poor and better-off women. Women were engaged through visual learning and participatory tools and learned from the facilitator and each other.

	Houweling, T. A. J. (2019)[55]		Indian study site, 80% of the population were from scheduled castes or tribes. In the Malawi study site, 95% were Christian and 88% were of Chewa ethnicity. The powerful position of the mother-in-law and other family members in overseeing the behaviour of daughters-in-law, particularly newlyweds, was emphasised at Asian sites. In Bangladesh, cultural factors prevented women from going outside the home to gain information and overcome. superstitions around birthing.	from each other and shared equally among better-off and poor families and there were no discrimination discussions.	Groups provided a social support network that addressed some financial barriers to care and gave women the confidence to promote behaviour change. Information was disseminated through home visits and other strategies. The social process of learning and action, which led to increased knowledge, confidence to act, and acceptability of recommended practices, was key to ensuring behaviour change across social strata. These equitable effects were enabled by the accessibility, relevance, and engaging format of the intervention. **Facilitation** The group facilitators were local women who convened women's groups. The Facilitators were viewed as a respected source of information and knowledge. Facilitators enabled inclusion of all socioeconomic strata, ensuring that strategies were low-cost, and that discussions and advice were relevant. "The facilitator sits with us and makes us understand". **Activities** Home visits to pregnant women who were not part of the groups to tell them about safe birthing practices and newborn care, the distribution of safe delivery kits, and stretcher schemes to bring women in labour to the hospital. **Decision-making** In accordance with PLA cycle **Reflexivity** The difficulties around the cultural "ke garne" raised issues around non-Nepali's understanding of culture and the non-Nepali's facilitation role.
25	Morrison, J., Thapa, R., Sen, A., Neupane, R., Borghi, J., Tumbahangphe, K. M., . . . Costello, A. (2010)[41]	Community women	**Community/culture** The 'middle poor' – attended and literate, salaried women were less likely to attend, as were those of the most marginalized ethnic group. Fifteen different ethnic groups in Makwanpur, the largest of which were Tamang, then Brahmin/Chhetri. Caste discrimination still a powerful indicator of social inclusion and empowerment **Institutional/ political** Low-quality services, limited geographical access, and poor infrastructure constrains health service use in rural areas.	Intra group relationships were important in management and utilization of funds. Mutual trust and understanding between friends were important for funds success. Outsiders had to convince group members of the validity of their claims and their credit worthiness.	**Facilitation/structure/process** The intervention consisted of 109 women's groups convened by locally employed female facilitators. Facilitators led the groups through a participatory action cycle of problem prioritization and community planning to address salient local neonatal and maternal health problems. **Decision-making** Community women with facilitation. PLA approach **Reflexivity** Unclear

26	Morrison, J., Thapa, R., Hartley, S., Osrin, D., Manandhar, M., Tumbahangphe, K., . . . Costello, A. (2010)[40]	Community women	**Community/culture** Different ethnic groups, arranged marriages, mother in laws are in control. Women busy with home and farming. Ethnicity and caste were important. Ethnic discrimination was evidenced but not explicitly discussed. Homogeneity of ethnicity was a hindrance to social and economic development. In ethnically mixed communities villagers discuss and work for change. **Political/Institutional** Insurgency created insecurity. National strikes and army patrols sometimes restricted access to health services. Mistrust of local government health services was ubiquitous, and all respondents expressed their dissatisfaction. Some agencies were seen as trustworthy (including MIRA funds). **Approach to CP** Developmental Instrumental	Group felt to be a source of support and place for learning and support, a place for learning and sharing knowledge	**Structure** Key features were an organisational commitment to a participatory learning and action approach (PLA), and the overall aim of women's empowerment. **Facilitation** In Nepal, women met monthly, and a facilitator led them through a participatory action cycle. **Decision-making** PLA approach **Reflexivity** Aware for the need of local interviewers, translating interviews into Nepal first, social scientist with good spoken Nepali and aware of cultural differences
27	Morrison, J., Tamang, S., Mesko, N., Osrin, D., Shrestha, B., Manandhar, M., . . . Costello, A. (2005)[2]	Local women	**Community/ culture** Diverse cultural groups. Some groups were dominated by women from higher castes, but these higher castes served as a stimulant to \traditionally subservient ethnic groups. Difficulties in linking problems to strategies due to the cultural phenomenon	The continuing activity of most groups suggests that usually group members found the experience useful and enjoyable. Women actively participated in	**Structure** The facilitator used a meeting manual, adapted from the Warmi project, to guide the women's groups through problem identification and community planning using participatory iterative methods. Each facilitator leading nine groups per month, covering an average population of 7000. **Facilitation** Meetings were facilitated by a paid, locally based woman, who was selected on merit and trained in facilitation techniques. They worked with the local unpaid Female Community Health Volunteer. **Activities**

			of 'ke garne' a belief in fatalism - the feeling that "one has no personal control over one's life circumstances, which are determined through a divine external agency" Experience was that fatalism affected the way people viewed themselves in relation to a problem, and also the power and capacity they believed themselves to have in overcoming it.	learning together and gathered much information from their communities.	Strategies discussed during planning together and successfully implemented were the mother and child health fund, locally produced clean home delivery kits, management and production of stretchers, and awareness raising through video shows. **Decision-making** PLA approach **Reflexivity** Acknowledgement of the cultural phenomenon of “ke garne” that affected women's role in the study and facilitation.
28	Lugo, N. R. (1996)[30]	Pregnant women (identified as high risk)	**Community/culture** Differences in culture of urban and rural areas with rural areas more receptive to the program. Hispanics in rural area had minimal previous contact with social service agencies and community-based programs and so may have been more receptive to the program. **Institutional** This program was funded, in part, as an alternative model for providing state-mandated case management for at-risk pregnant women. The funding guidelines played a very strong role in the day-to-day functioning of the program.	The groups did develop a sense of community among participants, encouraging mutual support and problem solving. Relationships in the groups carried over beyond the group.	**Structure** Peer support groups were to provide a safe, reaffirming forum for discussing issues that participants feel are important. Some group members demonstrated a strong sense of ownership of the group meetings, taking over leadership and assigning group tasks among themselves. **Facilitation** The facilitator -hired women who were peers, in one way or another, of the women in the neighbourhoods the program had targeted. **Decision-making** Collective problem solving keeping to the principle of women being able to define their health needs and choose priorities. **Reflexivity** Unclear
29	Dongre, A.R., Deshmukh, P.R. & Garg, B.S (2009)[29]	Pregnant women	**Institutional/political** Village Coordination communities (VCC) raised health funds from villagers for village level health activities.	Unclear – not reported on	**Structure** In each village, monthly comprehensive, participatory, maternal and child health services and group health education sessions were delivered through a health personnel team including a VCC member. **Activities** Social workers ensured group members had information regarding newborn danger signs during their monthly village-based meetings.

					VCC village health and emergency transport plan developed. **Decision-making** Social worker facilitated VCC to develop village health plan and act on priorities **Reflexivity** Unclear
30	Dickinson, P. & and Joe, T. (2010)[31]	Young mums and pregnant young women-mostly Maori or from Pacific Island Nations	**Community culture/institutional/ political** Not mentioned	The program successful in connecting young women to a group of peers in similar circumstances	**Structure** The program objectives were to: encourage self-value; create individualised developmental plans for each young mother; make connections so that young mothers are supported to seek community organisations to meet their visions; provide role models and mentoring support from experienced mothers and child carers; and to meet other young mothers and reduce isolation. **Facilitation** A blend of structured and unstructured (self-directed by the young mums with support from facilitator) The designated facilitator was the Young Mums Programme Coordinator based at Te Waipuna Puawai. **Decision-making** Women and facilitators **Reflexivity** Unclear
31	Tripathy et al. (2016)[42]	Study participants were women of reproductive age (15–49 years) who gave birth between Sept 1, 2009, and Dec 31, 2012.	**Community/culture** Before randomisation, we obtained permission from local community representatives (village headmen in Jharkhand and Panchayati Raj institution leaders in Odisha) to work with women's groups and ASHAs, and to collect data in their areas. Village head men were involved in randomisation. The proportion of Adivasis (Indigenous tribes) within the study clusters was 58–70%. 70% of facilitators ASHA were Adivasi. **Institutional** One of the study districts (Khunti, Jharkhand) had experienced severe law and order problems during the baseline and evaluation periods. "In one cluster, we could	Unclear about intra group relationships amongst women	S**tructure** Meetings were held fortnightly for the first four months (phases 1 and 2), and every month thereafter. **Process** The intervention was a four phase PLA cycle of women's group meetings led by ASHAs. In the first phase, ASHAs helped the groups identify and prioritise maternal and newborn health problems using picture cards and a participatory voting game. In the second phase, groups listened to stories with local motifs featuring the causes of their prioritised problems and potential solutions, held a community meeting seeking support of other community members. In phase 3, the groups implemented their chosen strategies and learned about other practical actions to improve maternal and newborn health. Finally, in phase 4, the groups evaluated the meeting cycle and progress against their strategies. **Facilitation** Was by ASHAs, (70 % Adivasi) The 152 ASHAs were typical of ASHAs working in the study area; they had undergone at least three government training sessions and were all village based. ASHAs' motivation and earnings were strongly influenced by the support and recognition given to them in the community. Each coordinator employed by Ekjut supervised ten ASHA through bimonthly meetings in the first 3 months of the intervention, and meetings every month thereafter. ASHAs were given an incentive of INR 100 (US$2) during training and INR. 200 for each woman's group meeting. **Strengthening local health systems** In both intervention and control areas, attempt were made to carry out at least one village health sanitation and nutrition committee meeting about rights and entitlements per village. **Decision-making** PLA approach **Reflexivity** Unclear

			not do any data quality checks and the Board voted to exclude it from the final trial analyses at its final meeting in December 2013.		
32	Morrison, Tumbahangh e, Sen, et al. (2020)[32]	All women in the communities were potential members	**Community/ cultural** 43 geopolitical VDCs 83% population engaged in agriculture. Almost half Tibeto-Burman descent Difficulties implementing first PLA cycle strategies arose because FCHVs not used to facilitating discussing as usual role is information giving. **Institutional/political** Federalism and decentralisation of Nepalese health system at end of 2017. National political focus on achieving MDG 5 (reducing maternal and child mortality) through skilled attendance and institutional delivery. Increase in number of trained health workers but policy incentive was for institutional births not home deliveries.	Unclear about intra women's groups relationships	**Structure** Facilitators ran 203 women groups per month. Eight groups ran twice monthly. Timing of meetings was according to community convenience. **Facilitation** Was by 195 FCHVs trained by researchers in facilitation skills, PLA process and how to run meetings. Monthly supervision meetings were limited due to geography, remoteness, number of groups to supervise and ad hoc nature of PWGs. **Activities** Groups identified strategies to address barries to institutional delivery and implemented strategies in two PLA cycles. Printed manual contained discussion points, games and stories to assist discussion. **Health system strengthening** Health Management Committee strengthening intervention using Appreciative Inquiry was also implemented although groups did not meet regularly. FCHV attended health strengthening sessions. **Decision-making** PLA approach **Reflexivity** Awareness of culture of didactic health information
33	Ronaasen, Steenkamp, Williams, Finnemore, & Feeley (2020)[33]	Mothers and caregivers of young children from purposively chosen communities association with local Early Childhood Development centres (ECD)	**Community/ cultural** Consistently high prevalence of HIV infection in Eastern Cape, with impoverished communities experiencing interplay among biological, socio-behavioural and contextual factors including stigma. Child rearing practices affected by traditional beliefs of family.	Outcomes indicated that women received peer support from other group members	**Structure** Parent support group based at the ECD centre. **Facilitation** Mentor mothers (30) engaged with skills training materials, established program called "Sakha Esethyu: It starts with us we are building our own". Shared learning approach. Mentors received support through indigenous social workers using local language. On-site mentors **Activities** Conversations around reduction of HIV risk in both HIV positive and negative groups, PMTCT, antiretroviral drug adherence and breastfeeding support and understanding child health record. **Decision-making** Unclear **Reflexivity**

					Unclear
34	Sharma, S., Mehra, D., Akhtar, F., & Mehra, S. (2020).[56]	Women 15-35 from marginalised sections of society	**Community/cultural** Women's groups had members belonging to scheduled caste, scheduled tribe and below the poverty line. Cultural and religious views were deep rooted. Districts targeted were those falling under the category of low or very low human development index.	Unclear	**Structure** Structured two-hour community sessions held on 10 MCH topics and home visits. Groups had 23-27 women members. Training module and flip charts in Hindi were used. **Facilitation** Peer educators (1,500) were selected from the women's group and trained by outreach workers for four days in ten topics related to MCH and financial literacy. Education tools were used. **Activities** Peer educator led groups discussing ten MCH set topics. Linkages made by outreach workers between women and the Mahatma Ghandi National Rural Employment Guarantee Act (MGNREGA). Outreach workers also sensitised health workers about access to health services for poor women. Orientation meetings with community institutions concentrating on MCH village health plans. **Decision-making** Unclear **Reflexivity** Unclear
35	Colbourn,T., Nambiar, B., Bondo, A., et al (2013) [34]	All pregnant women in surveillance areas who agreed to take part were eligible and enrolled if they became pregnant.	**Community/cultural** Not linked to groups but poverty, poor MCH knowledge, illiteracy	Unclear	**Structure** Structured group program with 8 sessions of planning and then 4 other steps in implementation and 4 steps in evaluation. Each facilitator formed nine village women's groups **Facilitation** Volunteer facilitators, supported by nine staff. Volunteers followed a set program and women made decisions about local issues and implemented action. **Activities** Health education, voluntary testing and counselling for HIV/AIDS, village savings and loans, bed nets, vegetable gardens and bicycle ambulances. **Decision-making** Women in the groups **Reflexivity** Unclear